

# A360 Agent Exchange

Full listing catalog — 16 marketplace agents, complete copy, pricing, KPIs, and media manifests

All ratings, install counts, and reviews in this catalog are demo placeholder data prepared for a realistic marketplace mockup. Replace with real figures before public launch.

|                   |                                                                           |
|-------------------|---------------------------------------------------------------------------|
| Prepared for      | Aesthetics360 Agent Exchange                                              |
| Prepared date     | July 2026                                                                 |
| Listings included | 16, across 6 categories                                                   |
| Use case          | Content source for exchange listing pages; stakeholder and partner review |

This report is a planning and strategy document produced by AI-assisted research. Figures, vendor capabilities, and compliance postures require direct diligence before contracting or public use.

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## Category: Consult Room — Full Listings

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Agents that live inside the consultation: capture it, coach it, visualize it, finance it. All ratings, installs, reviews, and proof stats are **DEMO DATA** for mockup purposes.

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# 1. Sloane — Consult Coach FLAGSHIP

**Slug:** sloane-consult-coach · **Category:** Consult Room · **Built by A360 Badges:** Built by A360 A360 Verified HIPAA BAA EMR-Neutral Staff Pick **Tags:** consult-intelligence sales-coaching roi-tracking upsell kpi documentation

## Card

- **Tagline:** Every consult, scored. Every missed dollar, found.
- **Short description:** Records and analyzes your consultations, scores them against proven closers, and shows the revenue you didn't ask for.
- **Price from:** \$499/mo per location · 14-day free pilot · Month-to-month
- **Rating:** ★ 4.9 (127) — DEMO · **Installs:** 340+ practices — DEMO

## Hero

- **Headline:** Your best closer, in every room.
- **Subheadline:** Sloane listens to every consultation, scores it like a sales coach, prices every opportunity that walked out the door, and hands your team the playbook to close it next time.
- **CTA:** Start free pilot → · Secondary: Watch 2-min demo

## Overview

The consultation is where your practice makes or loses its money — and until now, it's been a black box. Sloane changes that. With patient consent, Sloane captures each consultation and turns the conversation into intelligence: which treatments the patient asked about, which objections stalled the close, what they said they'd do "after the wedding" or "next spring," and what was never offered at all.

Every consult gets a Consult Score across the behaviors that separate your best closer from the rest: treatment-plan presentation, price confidence, financing mention, package framing, and follow-up commitment. Every un-booked opportunity gets a dollar value, mapped to your actual service catalog — not a generic estimate. Your providers get a private weekly coaching digest. You get a practice-wide view of consult-to-close by provider, by treatment, by objection type.

Field-sales teams using this exact method report 20–36% close-rate lifts. Nobody has brought it into the aesthetic consult room — until Sloane. HIPAA-native, consent-first, and it works no matter which EMR you run.

## Key features

- **Consult Score** — every consultation graded on 9 close-critical behaviors, benchmarked against your practice's own best outcomes
- **Missed-Revenue Ledger** — every treatment discussed-but-not-booked, priced from your catalog, with the reason it stalled

- **Future-Intent Tracker** — "after my wedding in October" becomes a dated, priced follow-up — automatically
- **Provider Coaching Digests** — private, weekly, specific: "You offered financing in 2 of 11 consults; your close rate doubles when you do"
- **Objection Analytics** — see whether price, fear, timing, or spouse is what's really killing your closes
- **Consent-first capture** — built-in recording-consent workflow, two-party-state aware, patients can opt out at any time
- **Works with your EMR** — Boulevard, Zenoti, Aesthetic Record, PatientNow, Symplast, or CSV

## How it works

1. **Capture** — a room device or the provider's iPad records the consult after one-tap patient consent.
2. **Extract** — Sloane's engine (A360's V3.1 extraction) structures interests, objections, quotes, and future intents, mapped to your catalog with dollar values.
3. **Score & coach** — each consult is graded; providers get private digests; managers get trends.
4. **Recover** — un-booked opportunities flow to Ember (Reactivation) and Fina (Financing) to be closed — with attribution back to the original consult.

## KPIs moved

- **Consult-to-close rate** (analog benchmark: +20–36%)
- **Average ticket** (package/financing framing lift)
- **Treatment-plan acceptance rate**
- **Revenue per consult hour**

## Proof bar (DEMO)

\$11,400 avg. missed revenue surfaced per location in first 30 days · +27% consult-to-close in 90 days · 9 of 10 providers improve score within 6 weeks

## Integrations

Boulevard · Zenoti · Aesthetic Record · PatientNow · Symplast · CSV import · Ember · Fina · Ledger

## Pricing

| Tier     | Price                 | Includes                                                        |
|----------|-----------------------|-----------------------------------------------------------------|
| Solo     | \$499/mo              | 1 location, up to 3 providers, 100 consults/mo                  |
| Practice | \$799/mo              | 1 location, unlimited providers & consults, objection analytics |
| Group    | \$699/mo per location | Multi-location benchmarks, MSO rollup dashboard                 |

## FAQs

1. **Is recording consultations legal?** Yes, with consent. Sloane ships with a consent workflow (verbal script + e-sign) configured for your state, including all-party-consent states like CA, FL, WA, and IL. No consent, no recording — enforced in software.
2. **Is this HIPAA compliant?** Yes. BAA included at every tier, recordings encrypted at rest and in transit, and your data is never used to train shared models.
3. **Will my providers feel surveilled?** Coaching digests are private to each provider by default. Managers see scores and trends, not raw audio, unless you enable full review.
4. **Do I need to change my EMR?** No. Sloane is EMR-neutral. Catalog and calendar sync make it smarter, but a CSV export is enough to start.
5. **What happens to the audio?** Retention is configurable (30/90/365 days). Transcripts and structured data persist; audio can auto-delete.
6. **Can I export my data if I leave?** Always, free, in standard formats. Month-to-month means we earn you every month.

## Sample reviews (DEMO — fictional)

★★★★★ "We found \$23K in un-offered treatments in the first month. Our newest injector's close rate went from 31% to 52% in eight weeks." — *Practice owner, 2-location medspa, Dallas TX* ★★★★★ "The future-intent list alone pays for it. Patients who said 'maybe in the fall' actually hear from us in the fall — with the exact treatment they asked about." — *Practice manager, plastic surgery, Scottsdale AZ* ★★★★★☆ "Setup took a week because of our consent forms, but support rewrote them with us. Now it's invisible." — *Medical director, dermatology, Chicago IL*

## Compliance block

HIPAA BAA included · SOC 2 Type II infrastructure · No PHI used for model training · Consent-gated recording, state-aware · Data export always free

## Media manifest

- **Icon:** rounded-square, deep teal, minimalist speech bubble containing an upward tick; "S" monogram variant
- **Hero image:** split-screen — left: warm consult room (provider + patient, iPad on lap); right: Sloane's Consult Score panel floating on dark UI. Style: premium SaaS, soft depth of field.
- **Screenshot 1 — Consult Score card:** single consult view; score ring 82/100; behavior checklist (✓ plan presented, ✗ financing offered); sentiment strip along an audio timeline
- **Screenshot 2 — Missed-Revenue Ledger:** table of opportunities: "Neck contouring — \$3,400 — Objection: price — Intent: Oct (post-wedding)" with Recover buttons
- **Screenshot 3 — Provider leaderboard:** consult-to-close % by provider with trend sparklines and coaching-focus chips

- **Screenshot 4 — Weekly digest (mobile):** push-notification style coaching card
  - **Video (30s) script:** [0–4s] Consult room, natural audio. "Every consult is a sales conversation." [4–10s] UI: transcript lines light up — *interested: lip filler; objection: price; intent: October.* [10–18s] Score ring animates; ledger totals \$11,400. [18–25s] Digest hits provider's phone: "Offer financing earlier — your close rate doubles." [25–30s] Logo. "Sloane. Your best closer, in every room. On the A360 Exchange."
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## 2. Quill — Aesthetic Scribe

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**Slug:** quill-aesthetic-scribe · **Category:** Consult Room · **Built by A360 Badges:** Built by A360  
 A360 Verified HIPAA BAA EMR Sync **Tags:** scribe documentation charting consult-  
 intelligence compliance

### Card

- **Tagline:** The scribe that pays for itself in found revenue.
- **Short description:** Aesthetic-native AI charting — units, sites, lots, consents — that also feeds every consult into your revenue engine.
- **Price from:** \$149/provider/mo · 14-day free pilot
- **Rating:** ★ 4.8 (98) — DEMO · **Installs:** 290+ practices — DEMO

### Hero

- **Headline:** Chart in 40 seconds. Not 40 minutes after close.
- **Subheadline:** Quill listens, writes the aesthetic-grade note — units injected, sites, lot numbers, GFE language, consent confirmations — and hands the conversation to your growth agents.

### Overview

Generic medical scribes weren't built for aesthetics. They don't know a syringe from a session, can't structure units-per-site, and leave your Good Faith Exam language to chance. Quill is aesthetic-native: it produces charts your medical director will co-sign — treatment areas, product, units, lot and expiry, technique, aftercare given, consent confirmed — formatted for your EMR.

Then Quill does what no other scribe does: the same conversation flows into A360's extraction engine, so the interests, objections, and future intents inside the visit become fuel for Sloane, Ember, and Fina. One recording; documentation *and* growth. Providers report 6–10 hours a week back; the found-revenue side means Quill is the rare scribe with a payback measured in bookings, not minutes.

### Key features

- Aesthetic-native note templates: injectables (units/site/lot), laser (settings/passes), body contouring, GLP-1/wellness visits
- GFE- and consent-aware: flags missing Good Faith Exam language or unconfirmed consents before the chart closes
- One-tap co-sign packet for medical directors, with audit trail
- Photo prompts: reminds the provider to capture before/after at the right moments
- EMR write-back: Boulevard, Aesthetic Record, PatientNow, Symplast (others via export)
- Feeds Sloane/Ember automatically — documentation and revenue from a single capture

## How it works

1. Tap record (consent captured once per patient, reused per policy).
2. Treat and talk normally — Quill structures the note in real time.
3. Review the draft in ~40 seconds, sign, and write back to your EMR.
4. Growth signals route to your other A360 agents automatically.

## KPIs moved

- Documentation time per visit (target: -80%)
- Chart-compliance rate (GFE/consent completeness)
- Same-day chart closure rate
- Downstream: extraction-driven revenue per visit

## Proof bar (DEMO)

38 sec avg. review time · 100% charts with GFE language · 6.4 provider-hours saved weekly

## Pricing

| Tier          | Price             | Includes                                            |
|---------------|-------------------|-----------------------------------------------------|
| Provider      | \$149/provider/mo | Unlimited visits, EMR write-back                    |
| Practice      | \$499/mo          | Up to 6 providers, co-sign workflows, photo prompts |
| + Growth Link | +\$99/mo          | Streams extraction to Sloane/Ember/Fina             |

## FAQs

1. **Which EMRs do you write into?** Boulevard, Aesthetic Record, PatientNow, and Symplast today; everything else via formatted export. Reads are broader than writes.
2. **Does it handle injectable lot tracking?** Yes — spoken lot numbers are structured, validated against your Vial inventory (if installed), and flagged if near expiry.
3. **Is my audio training your AI?** No. Your PHI never trains shared models. BAA at every tier.
4. **What about my medical director's co-sign?** Quill batches charts into a co-sign queue with per-chart audit trails — built for multi-state MSO compliance.
5. **Can I use Quill without the marketing agents?** Absolutely. Growth Link is optional and off by default.

## Sample reviews (DEMO — fictional)

★★★★★ "I used to chart at my kitchen table at 9pm. Now I co-sign before the patient reaches the parking lot." — NP owner, medspa, Nashville TN  
 ★★★★★ "The GFE flagging alone survived our compliance audit. The found-revenue link is the bonus nobody else has." — MSO clinical director, 9 locations

## Compliance block

HIPAA BAA · SOC 2 Type II · No training on PHI · Configurable audio retention · State-aware consent

## Media manifest

- **Icon:** feather quill forming a checkmark, ink-blue on ivory
  - **Hero:** provider mid-treatment (gloved hands, syringe) with a translucent structured note assembling itself beside them
  - **Screenshots:** (1) live note view with unit/site table filling in; (2) co-sign queue; (3) compliance flags panel ("GFE language missing — insert template?"); (4) Growth Link toggle showing signals flowing to Sloane/Ember
  - **Video (30s):** [0–5s] Provider treating, talking naturally. [5–12s] Note assembles: "Glabella — 20u — Lot B2214". [12–18s] "Review: 38 seconds." Sign. EMR write-back animation. [18–25s] Toggle: "Growth Link ON" — signals stream to Ember. [25–30s] "Quill. The scribe that pays for itself. On the A360 Exchange."
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### 3. Vista — Outcome Visualizer

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**Slug:** vista-outcome-visualizer · **Category:** Consult Room · **Verified Partner** (white-label) **Badges:** Verified Partner A360 Verified HIPAA BAA New **Tags:** simulation visualization before-after upsell consult-intelligence

#### Card

- **Tagline:** Show the after. Close the before.
- **Short description:** Medically accurate AI outcome simulation for injectables and facial treatments — in the consult and on your website as a lead magnet.
- **Price from:** \$299/mo per location
- **Rating:** ★ 4.7 (64) — DEMO · **Installs:** 180+ practices — DEMO

#### Hero

- **Headline:** Patients don't buy syringes. They buy the mirror.
- **Subheadline:** Vista generates realistic, provider-controlled previews of filler, tox, and contouring outcomes — in seconds, on an iPad, no 3D camera rig.

#### Overview

Hesitation is the most expensive thing in your consult room. Vista removes it visually: a front-facing photo becomes a medically plausible preview of the discussed treatment plan — conservative by design, adjustable by the provider, never oversold. Practices using outcome visualization report double-digit consult-conversion lifts, and Vista's embeddable web widget turns your site into a lead engine: visitors upload a selfie, see a preview, and book a consult (contact-gated, consent-first).

Vista pairs naturally with Sloane — when a simulated treatment isn't booked, the simulation and the extracted objection travel together into your follow-up, so the patient's "someday" email shows *their own face*, not stock art.

#### Key features

- Photorealistic simulation for lips, cheeks, chin, jawline, tox lines, and profile balancing — provider-adjustable intensity
- iPad-native; no hardware rig; results in under 60 seconds
- Website lead-magnet widget with consent + contact gate (captures 120+ qualified leads/weekend in vendor case studies)
- Every simulation linked to the consult record and follow-up sequence
- Media-release management: e-sign consent attached to every image
- Guardrails: anatomically conservative models; no "unrealistic result" mode

## KPIs moved

- Consult-to-treatment conversion (visualization analog: +15–30%)
- Web lead volume and lead-to-consult rate
- Average plan value (multi-area plans visualized together)

## Pricing

| Tier          | Price    | Includes                                   |
|---------------|----------|--------------------------------------------|
| In-Room       | \$299/mo | Unlimited consult simulations, 2 iPads     |
| In-Room + Web | \$499/mo | Adds website widget, lead routing to Piper |

## FAQs

1. **Is this the same engine as [white-label partner]?** Vista is powered by an A360 Verified partner engine and operated under A360's BAA and PHI controls — one contract, one compliance standard.
2. **Can results over-promise?** No. Intensity is provider-capped, previews are watermarked "simulation," and every image carries a disclaimer configured to your med-board's advertising rules.
3. **Who owns the images?** Your practice. Patients consent per use (consult vs marketing) — separately.
4. **Does it work for darker skin tones?** Yes — the engine is validated across Fitzpatrick I–VI; we publish the validation summary.

## Sample reviews (DEMO — fictional)

★★★★★ "Our web widget booked 41 consults in its first month. Patients arrive already sold — they've seen it." — *Owner, medspa, Miami FL* ★★★★★☆ "In-room close rate on filler plans is up ~20%. Wish it did body sim too — told it's on the roadmap." — *PA, plastic surgery, Newport Beach CA*

## Compliance block

HIPAA BAA · Consent per simulation · BIPA/biometric-statute aware (IL/TX/WA) · Media-release e-sign built in

## Media manifest

- **Icon:** circular before/after split face silhouette, lilac gradient
- **Hero:** patient holding iPad seeing her simulated result; provider beside her; UI slider "Conservative ► Enhanced"
- **Screenshots:** (1) before/after slider UI; (2) multi-area treatment plan with per-area pricing; (3) web widget on a practice site; (4) consent + media release screen
- **Video (30s):** [0–5s] Patient: "I just can't picture it." [5–15s] iPad capture → simulation slider moves. Patient's face lights up. [15–22s] Plan panel: 3 areas, \$2,850, "Book now / Finance with Fina." [22–30s]

"Vista. Show the after. Close the before."

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## 4. Fina — Financing Concierge

**Slug:** fina-financing-concierge · **Category:** Consult Room · **Built by A360 Badges:** Built by A360  
 HIPAA BAA EMR-Neutral Outcome Pricing **Tags:** financing bnpl upsell no-show-reduction  
 consult-intelligence

### Card

- **Tagline:** Sticker shock, handled.
- **Short description:** Detects price hesitation, orchestrates financing across Cherry, PatientFi & CareCredit, and follows up until the plan is affordable.
- **Price from:** \$199/mo + \$10 per financed booking (capped)
- **Rating:** ★ 4.8 (71) — DEMO · **Installs:** 210+ practices — DEMO

### Hero

- **Headline:** "It's a lot right now" isn't a no. It's a workflow.
- **Subheadline:** Fina waterfalls every hesitant patient across your financing partners for the best approval, sends pre-qualification links at the right moment, and coaches your team to offer financing before the objection lands.

### Overview

Financing raises average tickets and close rates — but only when it's actually offered, and most consults never mention it. Fina fixes both sides. In the room (via Sloane) it nudges providers when price hesitation appears. After the consult, it sends a soft, compliant pre-qual link — no hard credit pull — and routes the application across Cherry, PatientFi, and CareCredit in the order most likely to approve *this* patient, lifting total approval rates without adding a single new lender agreement beyond the ones you already have.

Fina isn't a lender. It's the orchestration layer nobody built: approvals up, financing attach rate up, and a clean ledger showing financed revenue attributed to each consult and campaign.

### Key features

- Waterfall routing across your existing lender accounts (Cherry, PatientFi, CareCredit, Alphaeon) — best-fit first, automatic fallback
- Hesitation triggers from Sloane transcripts ("price," "spouse," "think about it") launch pre-qual follow-ups automatically
- "Financing offered %" tracked per provider — the classic close-rate driver, finally measured
- Deposit + financed-booking flows that cut no-shows
- Compliant messaging: no credit terms quoted by AI; soft-pull links only; lender disclosures automated
- Attribution: financed revenue tied back to consult, provider, and campaign in Ledger

## KPIs moved

- Financing attach rate (goal: 2–3x)
- Approval rate (waterfall lift)
- Average financed ticket vs cash ticket
- Consult-to-close on \$2K+ plans

## Pricing

| Tier  | Price                                           | Includes                                      |
|-------|-------------------------------------------------|-----------------------------------------------|
| Core  | \$199/mo + \$10/financed booking (cap \$500/mo) | All lenders, hesitation triggers, attribution |
| Group | \$149/mo per location                           | MSO reporting, 5+ locations                   |

## FAQs

1. **Is Fina a lender?** No. Fina orchestrates the lender accounts you already have. Your payouts, rates, and agreements don't change.
2. **Does the AI discuss credit terms?** Never. Fina sends patients to lender-hosted, compliant applications. It handles timing and follow-through, not terms.
3. **How is "financed booking" counted?** Only when a financed treatment lands on your calendar — auditable line-by-line in your Attribution ledger. That's what the \$10 fee bills against, capped at \$500/mo.
4. **What if we only use one lender?** Fina still lifts attach rate via timing and coaching, and adds a second lender in one click when you're ready.

## Sample reviews (DEMO — fictional)

★★★★★ "Financing attach went from 9% to 24% in one quarter. Same lenders. Same team. Just... offered every time now." — *Owner, medspa, Atlanta GA* ★★★★★ "The waterfall saved deals we used to lose to a single declined app." — *Consult coordinator, plastic surgery, Houston TX*

## Compliance block

HIPAA BAA · No AI-quoted credit terms · Lender disclosures automated · TCPA-compliant messaging with opt-out · Soft-pull pre-qual only

## Media manifest

- **Icon:** rounded coin with heartbeat line, emerald green
- **Hero:** patient at checkout smiling at phone showing "You're pre-qualified — \$3,400 plan from \$99/mo"; muted spa interior
- **Screenshots:** (1) waterfall board (Cherry → PatientFi → CareCredit, approval odds); (2) hesitation-trigger feed from Sloane; (3) "Financing offered %" per provider; (4) financed-revenue ledger



- **Video (30s):** [0–5s] Consult audio: "That's... a lot right now." [5–12s] Fina flags it; pre-qual text sends that evening. [12–20s] Approval screen; booking confirmed with deposit. [20–30s] Ledger: "+\$3,400 — financed — attributed to 6/14 consult." "Fina. Sticker shock, handled."

# Category: Front Office — Full Listings

All ratings, installs, reviews, and proof stats are **DEMO DATA** for mockup purposes.

## 6. Remi — Front Desk

**Slug:** remi-front-desk · **Category:** Front Office · **Built by A360 Badges:** Built by A360 A360  
 Verified HIPAA BAA EMR Sync Most Installed **Tags:** voice-ai sms booking deposits no-show-reduction spanish

### Card

- **Tagline:** Every call answered. Every night, weekend, and lunch rush.
- **Short description:** A 24/7 voice + text receptionist that quotes your real treatments and prices, books into your calendar, and takes deposits.
- **Price from:** \$349/mo per location · 14-day free pilot
- **Rating:** ★ 4.8 (203) — DEMO · **Installs:** 520+ practices — DEMO

### Hero

- **Headline:** 40% of your leads call after 6pm. Remi picks up.
- **Subheadline:** Remi answers every call and text instantly, quotes treatments from your actual catalog — not a generic script — books into your EMR, screens contraindications, and takes a card to hold the slot.

### Overview

Missed calls are the most expensive silence in your practice: roughly 40% of medspa inquiries arrive between 6pm and 8am, and callers who hit voicemail book with whoever answers next. Remi is an aesthetic-native receptionist, not a generic answering bot. It knows your menu because it's synced to your catalog — real treatments, real price ranges, real providers and availability. It books, reschedules, and cancels directly in your calendar, asks the pre-consult screening questions your clinical team requires, and takes deposits so 2am bookings still show up at 2pm.

Every conversation lands in your KPI ledger: calls answered, bookings made, revenue attributed. Practices moving from voicemail to 24/7 AI answering report answer rates jumping from ~60% to 99% and booking rates from ~36% to 52%. When a caller needs a human, Remi transfers warm, with context.

### Key features

- 24/7 voice + SMS + webchat, natural conversational speech, English + Spanish
- Catalog-true answers: treatments, price ranges, promos, prep/aftercare pulled from your synced menu
- Direct calendar booking with your EMR (Boulevard, Zenoti, Aesthetic Record, PatientNow) or booking-link handoff for others
- Contraindication pre-screening and new-vs-returning routing, configured by your clinical team
- Deposit capture at booking; automatic release on cancellation per your policy
- Warm human transfer with whisper summary ("Maria, new patient, asking about lip filler pricing, wants Thursday")

- Every call outcome logged to Ledger for attribution

How it works

1. Connect your calendar and catalog (30-minute guided setup).
2. Approve Remi's greeting, screening questions, and escalation rules.
3. Forward after-hours (or all) calls. Remi answers in under 2 rings.
4. Watch the morning digest: who called, who booked, what revenue was scheduled overnight.

KPIs moved

- Call answer rate (benchmark: 60% → 99%)
- Inquiry-to-booking rate (benchmark: 36% → 52%)
- After-hours revenue booked
- No-show rate (deposit effect)

Proof bar (DEMO)

99.2% answer rate · \$18,300 avg. after-hours revenue booked per quarter · -31% no-shows with deposits on

Pricing

| Tier        | Price                 | Includes                               |
|-------------|-----------------------|----------------------------------------|
| After-Hours | \$349/mo              | Nights/weekends coverage, 500 min      |
| Always-On   | \$599/mo              | 24/7 coverage, 1,500 min, Spanish line |
| Group       | \$499/mo per location | Central routing, MSO dashboard         |
| Overage     | \$0.15/min            | All tiers                              |

FAQs

1. **Will patients know it's AI?** Remi discloses it's a virtual assistant when asked and by default in its greeting — configurable within your state's disclosure rules. Most patients care that they got an answer at 9pm, not who gave it.
2. **What if a caller has a medical question?** Remi never gives medical advice. Clinical questions are captured verbatim and routed to your staff queue, with an optional urgent-escalation path.
3. **Can it really quote MY prices?** Yes — Remi answers from your synced catalog only. If it's not in your menu, Remi doesn't improvise; it offers a consult.
4. **What happens to recordings?** Encrypted, BAA-covered, retention configurable, never used to train shared models.

5. **We already have front desk staff.** Perfect — Remi takes the overflow, the after-hours, and the holds queue. Your team keeps the walk-ins and the relationships.

### Sample reviews (DEMO — fictional)

★★★★★ "First weekend live, Remi booked \$4,100 while we were closed. It paid for the year in two days." — *Owner, medspa, Frisco TX* ★★★★★ "Our front desk finally takes lunch. Callers can't tell, and honestly Remi is more consistent on pricing than we were." — *Practice manager, dermatology, Denver CO* ★★★★★☆ "Spanish support won us a whole patient segment. Wish voice cloning shipped sooner — told me Q3." — *Owner, medspa, San Antonio TX*

### Compliance block

HIPAA BAA · AI-disclosure configurable per state · No medical advice by design · TCPA-compliant outbound confirmations · Recording retention configurable

### Media manifest

- **Icon:** headset forming an "R", coral on cream
- **Hero:** dark dashboard showing a live call card ("Booking: Lip Filler consult, Thu 2:15pm, deposit \$50 ✓") over a nighttime spa storefront photo
- **Screenshots:** (1) live call transcript with booking card assembling; (2) morning digest email; (3) calendar with overnight bookings highlighted; (4) escalation queue with whisper summaries
- **Video (30s):** [0–4s] Phone ringing in a dark, closed spa. [4–12s] Remi answers; caller asks "how much is Botox?"; catalog card appears: "\$12/unit, Sarah has Thursday." [12–20s] Booking + deposit confirmation lands on the calendar. [20–30s] Morning: staff opens digest — "7 conversations, 3 bookings, \$1,240 scheduled." "Remi. Every call answered. On the A360 Exchange."

## 7. Piper — Lead Concierge

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**Slug:** piper-lead-concierge · **Category:** Front Office · **Built by A360 Badges:** Built by A360 HIPAA BAA EMR-Neutral **Tags:** speed-to-lead sms webchat booking no-show-reduction upsell

### Card

- **Tagline:** Reply in 60 seconds, or the lead books elsewhere.
- **Short description:** Instant multi-channel lead response, qualification, and pre-consult nurture — including the months-long follow-up nobody's team actually does.
- **Price from:** \$249/mo per location · 14-day free pilot
- **Rating:** ★ 4.7 (88) — DEMO · **Installs:** 260+ practices — DEMO

### Hero

- **Headline:** Leads contacted in 5 minutes convert 21x better. Yours wait 2 hours.
- **Subheadline:** Piper responds to every form fill, Instagram DM, and ad lead in under a minute — qualifies budget, timing, and treatment, books the consult, and nurtures the "not yet" pipeline for months so surgical-size cases don't slip away.

### Overview

Healthcare's average lead response time is over two hours; contact rates drop 71% after the first 30 minutes. Piper closes that gap to seconds — across webforms, Instagram DMs, Google leads, and paid campaigns — then does the part everyone skips: qualification and long-arc nurture. Piper asks the questions a great coordinator would (what treatment, what timeline, have you had it before, comfortable with the typical range?), books qualified leads straight to your calendar, and enrolls the rest in a patient, human-sounding sequence with before/afters, financing pre-qual via Fina, and check-ins timed to their stated intent.

Rhinoplasty leads convert in 6–18 months. Nobody's front desk follows up for 18 months. Piper does — politely, compliantly, and with every touch attributed so you finally know which campaigns produce consults, not clicks.

### Key features

- Sub-60-second response on webforms, IG/FB DMs, Google Business messages, and ad leads
- Conversational qualification: treatment, timeline, budget comfort, prior history — scored and routed
- Direct consult booking + reminder + show-rate sequence (before/afters, provider bio, parking, prep)
- Long-arc nurture for high-consideration cases (surgical, \$5K+ plans) — months of scheduled, intent-aware touches
- Instagram-native: comment-to-DM automation, story-reply capture
- Financing pre-qual handoff to Fina at hesitation signals
- Full source attribution into Ledger (campaign → lead → consult → treatment)

## KPIs moved

- Time-to-first-response (target: <1 min)
- Lead-to-consult conversion
- Consult show rate (nurture effect)
- Cost per booked consult by channel

## Proof bar (DEMO)

47 sec median response · +38% lead-to-consult · 84% show rate on nurtured consults

## Pricing

| Tier     | Price                 | Includes                                                  |
|----------|-----------------------|-----------------------------------------------------------|
| Core     | \$249/mo              | All channels, 300 leads/mo, booking + reminders           |
| Nurture+ | \$449/mo              | Unlimited leads, long-arc surgical nurture, IG automation |
| Group    | \$349/mo per location | MSO routing + rollups                                     |

## FAQs

1. **Does Piper sound like a bot?** Judge for yourself in the demo — short messages, natural cadence, no exclamation-mark spam. Every practice reviews and edits Piper's voice during setup.
2. **Which ad platforms connect?** Meta lead ads, Google Ads lead forms, TikTok lead gen, plus any webform via webhook or embed.
3. **Is texting patients compliant?** Yes — TCPA-compliant opt-ins, quiet hours enforced, opt-out honored instantly, PHI-safe messaging under our BAA.
4. **How is this different from Remi?** Remi answers inbound calls. Piper chases digital leads and nurtures the pipeline. They share one brain and one ledger — most practices run both.
5. **Can my team take over a conversation?** One tap. Piper hands off with full context and stays silent until re-invited.

## Sample reviews (DEMO — fictional)

★★★★★ "A mommy-makeover lead from last October booked in April because Piper never dropped the thread. That's a \$14K case our old process would've lost." — *Consult coordinator, plastic surgery, Nashville TN*

★★★★★ "IG comment-to-DM alone doubled our consult volume from social." — *Marketing lead, 3-location medspa, Phoenix AZ*

## Compliance block

HIPAA BAA · TCPA opt-in/opt-out enforced · Quiet hours by timezone · No medical advice · Full conversation audit log

### Media manifest

- **Icon:** paper-plane trailing a chat bubble, violet
- **Hero:** split phone screens — left: patient's IG DM "how much for lip filler?" answered in 47 seconds; right: practice dashboard lead card moving to "Consult booked"
- **Screenshots:** (1) lead inbox with response-time badges; (2) qualification chat thread; (3) long-arc nurture timeline (18 months, touches plotted); (4) channel attribution report
- **Video (30s):** [0–5s] Ad click at 11:48pm; form submitted. [5–12s] 11:49pm: Piper's text lands. Conversation qualifies the lead. [12–20s] Consult booked; nurture timeline animates for the "not yet" lead. [20–30s] Report: "This campaign: 31 consults, \$42K treated." "Piper. Speed wins. On the A360 Exchange."



# Category: Revenue Growth — Full Listings

All ratings, installs, reviews, and proof stats are **DEMO DATA** for mockup purposes.

### 3. Ember — Patient Reactivation STAFF PICK

**Slug:** ember-patient-reactivation · **Category:** Revenue Growth · **Built by A360 Badges:** Built by A360 A360 Verified HIPAA BAA EMR-Neutral Outcome Pricing Staff Pick **Tags:** reactivation recall sms voice-ai churn-prediction upsell

#### Card

- **Tagline:** Your next \$20K is already in your patient list.
- **Short description:** Treatment-cadence-aware win-back — Ember knows tox is a 90-day product and your patients' consult-room wishes, and re-books both.
- **Price from:** \$199/mo + \$9 per booked appointment (capped) · 14-day free pilot
- **Rating:** ★ 4.9 (156) — DEMO · **Installs:** 410+ practices — DEMO

#### Hero

- **Headline:** New patients cost \$285. Lapsed ones cost a text.
- **Subheadline:** Ember computes every patient's expected-return window by treatment — tox at 90 days, filler at 9–12 months, laser series on schedule — and runs warm, personal outreach that escalates from text to AI voice call until they're back on your calendar.

#### Overview

The average practice is sitting on hundreds of dormant patients — 30–50% of the database — while spending ~\$285 to acquire each new one. Reactivation is the highest-ROI motion in aesthetics, and it's also the one nobody's front desk sustains, because doing it right means knowing *when each patient is due for what*.

That's Ember's whole job. It builds a cadence model per patient per treatment from your EMR history (or a simple CSV), then runs the proven escalation: a friendly text, a second angle a week later, then a natural AI voice call — each personalized with the provider's name and the patient's actual treatment. With Sloane installed, Ember gets superpowers: it doesn't just know Amanda is 6 months lapsed — it knows Amanda said she wanted her neck done "after the wedding in October," and it reaches out in September with that exact treatment and a financing option. Recency tools send "we miss you." Ember sends the thing they already told you they wanted.

Dental-market analogs recover \$8K–\$25K/month per location at 10–20x ROI. Every recovered booking is attributed, line-by-line, in your ledger — which is also exactly what the per-booking fee bills against. If Ember doesn't book, Ember doesn't earn.

#### Key features

- Cadence engine: per-treatment return windows (tox ~90d, filler 9–12mo, laser/series schedules, memberships) computed per patient

- Intent-triggered campaigns (with Sloane): outreach built from consult-extracted interests, objections, and future-intent dates
- Text → text → AI voice escalation with human-warm scripts your team approves
- Books directly into your calendar; deposit optional
- Churn-risk scoring for membership patients before they lapse
- Do-not-disturb intelligence: caps, quiet hours, sentiment-based stop
- Campaign ledger: every message, cost, and recovered dollar attributed

**How it works**

1. Sync your EMR (Boulevard, Zenoti, Aesthetic Record, PatientNow) or upload a CSV — Ember builds the cadence map in minutes.
2. Approve tone, offers, and escalation rules.
3. Ember works the list daily, quietly, forever.
4. Review the weekly Recovered Revenue report; adjust with one click.

**KPIs moved**

- Rebooking / recall rate (analog: 15–25% of overdue patients per cycle)
- % of tox patients on-cadence
- Revenue per dormant patient
- Membership churn (predictive save-offers)

**Proof bar (DEMO)**

\$14,600 avg. monthly recovered revenue · 22% of dormant list reactivated in 90 days · 11.8x ROI median

**Pricing**

| Tier   | Price                                     | Includes                               |
|--------|-------------------------------------------|----------------------------------------|
| Core   | \$199/mo + \$9/booked appt (cap \$600/mo) | Cadence engine, SMS/email, ledger      |
| Voice+ | \$349/mo + \$9/booked appt (cap \$600/mo) | Adds AI voice-call escalation, 600 min |
| Group  | Custom                                    | MSO rollups, cross-location dedupe     |

*Booked appointments count only when confirmed on your calendar — auditable in your Attribution ledger.*

**FAQs**

1. **Is texting past patients allowed?** Yes, with proper consent and opt-out — Ember enforces TCPA rules, quiet hours, and instant opt-out automatically, under our BAA.

2. **Will it spam my patients?** No. Frequency caps, sentiment detection ("stop" or annoyance ends the sequence), and your approval on every campaign template.
3. **What data does it need?** Minimum: name, contact, last visit, treatment. More history = smarter cadence. CSV is fine to start.
4. **How do you count a "booked appointment"?** Confirmed calendar bookings sourced from an Ember conversation, matched by patient + time window, listed line-by-line in the ledger you can audit. Disputes are one click.
5. **Does it work without Sloane?** Yes — cadence-based reactivation stands alone. Sloane adds intent-triggered targeting, which is where the numbers get unfair.

### Sample reviews (DEMO — fictional)

★★★★★ "Month one: 41 patients back, \$19,700 booked. It found people from 2024 we'd completely forgotten." — *Owner, medspa, Charlotte NC* ★★★★★ "The voice calls shocked me — patients thanked US for checking in. Zero complaints, 14 bookings first week of Voice+." — *Practice manager, medspa, Sacramento CA* ★★★★★☆ "Wish the churn scores synced to my membership platform natively. CSV works fine meanwhile." — *MSO ops director, 6 locations*

### Compliance block

HIPAA BAA · TCPA-compliant with enforced opt-outs · Quiet hours by timezone · Consent state tracked per patient · No PHI in ad platforms

### Media manifest

- **Icon:** flame with a returning-arrow tail, ember orange
- **Hero:** dashboard "Recovered Revenue: \$14,600 this month" over a patient list re-igniting (grey rows warming to orange as they rebook)
- **Screenshots:** (1) cadence map — patients plotted by days-overdue per treatment; (2) an outreach thread escalating text → voice with the booking; (3) intent-triggered campaign card ("Neck contouring — said 'after wedding' — send Sept 3"); (4) recovered-revenue ledger
- **Video (30s):** [0–5s] "Meet the most profitable list in your practice: the one you already own." [5–13s] Cadence map lights up; texts fire; a voice call plays naturally. [13–22s] Calendar fills; ledger tallies \$14,600. [22–30s] Sloane tie-in: "She said October. Ember remembered." Logo. "Ember. On the A360 Exchange."

## 9. Halo — Membership Optimizer

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**Slug:** halo-membership-optimizer · **Category:** Revenue Growth · **Built by A360 Badges:** Built by A360 HIPAA BAA EMR-Neutral **Tags:** memberships loyalty churn-prediction kpi upsell

### Card

- **Tagline:** Your membership is a product. Halo is its product manager.
- **Short description:** Designs, prices, and continuously optimizes your membership from your actual visit data — churn predicted, breakage modeled, LTV proven.
- **Price from:** \$199/mo per location
- **Rating:** ★ 4.6 (54) — DEMO · **Installs:** 140+ practices — DEMO

### Hero

- **Headline:** Most memberships are priced by vibes. Yours won't be.
- **Subheadline:** Halo mines your visit and spend history to design tiers patients actually keep, predicts who's about to cancel in time to save them, and shows the number everyone claims: member LTV vs. non-member.

### Overview

Memberships work — member patients visit more and spend dramatically more (dental analogs: +76% visits, +146% procedures). But most practices copy a competitor's \$99/month plan and hope. Halo treats your membership like a product line: it models plan economics from your real data (what your patients already buy, at what cadence, at what margin), recommends tier structures and price points, projects MRR and breakage, and A/B tests offers. After launch, it watches usage patterns to flag members at churn risk — the ones whose visits are stretching — and triggers save-offers before the cancellation email arrives, not after.

Halo works alongside whatever membership platform or POS you run today; it's the intelligence layer, not another storefront.

### Key features

- Plan Designer: data-driven tiers, inclusions, and price points with margin and breakage modeling
- MRR forecasting and scenario compare ("\$149 two-tier vs \$99/\$199/\$299")
- Churn-risk scores per member with automated save-offer triggers (via Ember)
- Member vs. non-member LTV, visit frequency, and share-of-wallet reporting
- Compliance-aware structuring guidance (medical vs. cosmetic components, state rules)
- Works with your existing membership billing — no migration required

### KPIs moved

- Member churn rate

- MRR per location
- Member vs non-member LTV gap
- Membership attach rate at checkout

## Proof bar (DEMO)

–34% member churn in 6 months · \$21K avg. added MRR after redesign · 2.7x member vs non-member LTV

## Pricing

| Tier          | Price    | Includes                                     |
|---------------|----------|----------------------------------------------|
| Core          | \$199/mo | Designer, forecasting, churn scores          |
| + Save Engine | \$299/mo | Automated save-offers via Ember, A/B testing |

## FAQs

1. **Do I have to move off my current membership platform?** No. Halo reads your data and optimizes on top. It's Switzerland.
2. **How does it predict churn?** Visit-interval stretch, redemption slowdown, engagement drop — scored per member and validated against your own historical cancels.
3. **Can it design a first membership from scratch?** Yes — that's the funnest part. Give it 12 months of visit data and it drafts three plan structures with projected MRR by day two.
4. **Is a "medspa membership" even legal in my state?** Halo flags the structuring questions (medical vs. cosmetic components) and produces a summary for your healthcare attorney. Guidance, not legal advice.

## Sample reviews (DEMO — fictional)

★★★★★ "Halo told us our \$99 plan was leaving \$40/member on the table and our \$249 plan was priced into churn. Redesigned both. MRR up 38%." — *Owner, medspa, Austin TX* ★★★★★☆ "The churn saves are eerie. It flagged 12 members; 9 were genuinely about to cancel; we kept 7." — *Membership coordinator, 2 locations, Orlando FL*

## Compliance block

HIPAA BAA · Read-only access to billing data · Structuring guidance flags (not legal advice) · No PHI leaves your environment

## Media manifest

- **Icon:** ring/halo over ascending bars, gold on midnight
- **Hero:** Plan Designer screen comparing three tier structures with MRR projection curves

- **Screenshots:** (1) plan economics modeler; (2) churn-risk member list with save-offer buttons; (3) member vs non-member LTV report; (4) MRR forecast
  - **Video (30s):** [0–5s] "\$99/month. Why? ...Exactly." [5–15s] Halo ingests visit data; three plan options render with projections. [15–23s] Churn radar flags a member; save-offer sends; "Saved — \$2,340 LTV retained." [23–30s] "Halo. Membership, engineered. On the A360 Exchange."
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## 15. Ripple — Referral Engine

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**Slug:** ripple-referral-engine · **Category:** Revenue Growth · **Built by A360 Badges:** Built by A360  
 HIPAA BAA EMR-Neutral New **Tags:** referrals loyalty sms reviews upsell

### Card

- **Tagline:** Ask at the glow, not at random.
- **Short description:** Detects each patient's happiest moment and triggers the referral ask right then — two-sided rewards, tracked codes, zero PHI leakage.
- **Price from:** \$149/mo per location
- **Rating:** ★ 4.7 (47) — DEMO · **Installs:** 120+ practices — DEMO

### Hero

- **Headline:** Your happiest patient just left the building. Did anyone ask?
- **Subheadline:** Ripple times referral and review asks to satisfaction peaks — the glowing checkout comment, the "I love it" at follow-up — and turns them into tracked, two-sided referral offers your patients actually share.

### Overview

Referred patients close at the highest rate in aesthetics and cost essentially nothing to acquire — yet most practices' referral "program" is a dusty card by the register. Ripple productizes it. Using satisfaction signals (post-visit survey scores, five-star reviews, and — with Sloane — delight detected in conversation), Ripple sends the ask at the emotional peak, with a two-sided offer (\$50/\$50 is the proven default), a personal share link, and automatic reward fulfillment when the friend books. Referral capture lives outside your clinical records by design: no patient is ever identified as a patient to a third party, and shares carry marketing content only.

The dashboard finally answers what owners guess at: referral rate per 100 patients, revenue per referrer, your practice's true viral coefficient.

### Key features

- Peak-moment triggers: survey score  $\geq 9$ , 5-star review posted, consult-detected delight (via Sloane)
- Two-sided offers with personal share links/QRs and automatic reward issuance
- Ambassador tier: your top 5% of referrers get elevated rewards and early-access perks (FTC-compliant disclosures baked in)
- Review co-pilot: happy patients routed to Google; concerns routed to you first
- PHI firewall: referral system holds marketing consent data only — clinical data never leaves
- Attribution: referred-patient bookings and revenue in your Ledger



## KPIs moved

- Referral rate per 100 patients
- Referred-patient CAC (~\$0) and close rate
- Review velocity and average rating
- Revenue per ambassador

## Proof bar (DEMO)

6.2 referrals per 100 patients (from 1.1) · \$0 CAC on 78 referred bookings · +0.4 avg.

Google rating

## Pricing

| Tier        | Price    | Includes                                           |
|-------------|----------|----------------------------------------------------|
| Core        | \$149/mo | Triggers, two-sided offers, tracking               |
| Ambassador+ | \$249/mo | Ambassador tiers, review co-pilot, Sloane triggers |

## FAQs

1. **Is rewarding referrals legal for medical services?** Ripple's reward templates are configured for aesthetic/elective-cash contexts with state-rule flags, and rewards attach to retail/service credit patterns vetted for the vertical. Summary provided for your attorney; guidance, not legal advice.
2. **How is patient privacy protected?** The share link never reveals the referrer is a patient — it's a friend sharing a brand offer. Clinical systems and the referral system are separated by design.
3. **What stops reward gaming?** Booking-verified fulfillment, dedupe by contact fingerprint, and caps you control.
4. **Do ambassadors need FTC disclosures?** Yes, and Ripple auto-includes them in share content.

## Sample reviews (DEMO — fictional)

★★★★★ "We'd 'been meaning to build a referral program' for three years. Ripple shipped it in an afternoon. 31 referred bookings in Q1." — *Owner, medspa, Kansas City MO* ★★★★★☆ "The peak-moment timing is the trick — asks after a 5-star review convert 4x our old blast." — *Marketing manager, plastic surgery, Tampa FL*

## Compliance block

HIPAA-aware architecture (clinical/marketing separation) · FTC disclosure automation · State reward-rule flags · Opt-in messaging with instant opt-out

## Media manifest

- **Icon:** three concentric ripples from a heart, aqua

- **Hero:** phone showing "You made our day! Give a friend \$50, get \$50" over a happy post-treatment selfie moment (consented model)
- **Screenshots:** (1) trigger feed (review posted → ask sent); (2) share-link analytics; (3) ambassador leaderboard; (4) referred-revenue report
- **Video (30s):** [0–5s] Patient beams at follow-up: "I love it." [5–12s] Ripple detects the peak, sends the offer; she shares to a group chat. [12–22s] Friend books; both rewards auto-issue; ledger logs \$0 CAC. [22–30s] "Ripple. Ask at the glow. On the A360 Exchange."

## Category: Analytics & Attribution — Full Listings

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All ratings, installs, reviews, and proof stats are **DEMO DATA** for mockup purposes.

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## 2. Ledger — Revenue Attribution THE CONNECTIVE TISSUE

**Slug:** ledger-revenue-attribution · **Category:** Analytics & Attribution · **Built by A360 Badges:** Built by A360 A360 Verified HIPAA BAA EMR-Neutral Staff Pick **Tags:** attribution roi-tracking call-tracking kpi multi-location benchmarks

### Card

- **Tagline:** Which ad paid for that syringe? Now you know.
- **Short description:** HIPAA-safe attribution from ad click to treated revenue — the closed loop Google and Meta can't legally give you anymore.
- **Price from:** \$299/mo per location · 14-day free pilot
- **Rating:** ★ 4.9 (142) — DEMO · **Installs:** 380+ practices — DEMO

### Hero

- **Headline:** You spend \$285 to win a patient. Prove which dollar did it.
- **Subheadline:** Ledger joins your ad platforms, call tracking, forms, and EMR revenue into one compliant ledger — channel → lead → consult → treatment → LTV — so budget decisions stop being arguments.

### Overview

Attribution in healthcare broke on purpose: pixel restrictions on health data gutted GA4 and Meta reporting, and mainstream attribution tools won't sign a BAA. So practices guess — overspending on channels that look busy and cutting the ones quietly producing patients. Ledger rebuilds the loop compliantly. A HIPAA-safe first-party pixel, BAA-covered call tracking, form and DM capture, and your EMR's treatment ledger meet in one place, where every treated dollar traces back to its source — by channel, campaign, provider, location, and treatment type.

Ledger is also the proof layer for everything else on this Exchange: Ember's recovered revenue, Remi's after-hours bookings, Sloane's consult opportunities, Fina's financed tickets — all land here as auditable lines, not vendor claims. And with anonymized cross-practice benchmarks, you'll know if your \$285 CAC is a bargain or a bleed.

### Key features

- Full-funnel join: ad spend → click/call/DM → lead → consult → treatment revenue → LTV, per channel and campaign
- HIPAA-safe measurement: first-party pixel, PHI-free event schema, BAA-covered call tracking with recording controls
- Per-provider and per-location CAC, ROAS, LTV:CAC, and consult-conversion
- Agent proof ledger: every A360 Exchange agent's bookings/revenue itemized and auditable (this is where outcome pricing gets verified)

- Budget copilot: monthly reallocation recommendations in plain English
- Peer benchmarks: anonymized comparisons by region and practice size
- Board-ready exports and MSO rollups

### How it works

1. Connect ad accounts, phone lines, forms, and EMR (or nightly CSV) — guided, ~1 hour.
2. Ledger backfills 12 months where data allows.
3. Live dashboards populate within 48 hours; the first budget memo arrives in week two.

### KPIs moved

- Cost per acquired patient (benchmark ~\$285; top performers <\$200)
- LTV:CAC by channel
- Marketing-attributed revenue %
- Wasted-spend identified

### Proof bar (DEMO)

\$3,900 avg. monthly wasted spend identified · CAC -22% median at 6 months · 100% of agent outcomes auditable

### Pricing

| Tier | Price    | Includes                                           |
|------|----------|----------------------------------------------------|
| Core | \$299/mo | Full-funnel attribution, 2 phone lines, dashboards |
| Pro  | \$499/mo | Budget copilot, benchmarks, unlimited lines        |
| MSO  | Custom   | Cross-location, data-warehouse sync                |

### FAQs

1. **How is this HIPAA-compliant when Google Analytics isn't?** Ledger's pixel sends PHI-free, first-party events to A360's BAA-covered environment — never to ad platforms. Conversions shared upward are aggregated and de-identified per HHS guidance.
2. **Do you replace CallRail?** Ledger includes BAA-covered call tracking; if you love your current setup, we ingest it instead.
3. **Multi-touch or last-touch?** Both views, side by side, because the argument is the point. Default reporting is position-weighted with full transparency.
4. **Can my agency have access?** Yes — scoped seats with no raw PHI visibility.
5. **What's the "agent proof ledger"?** Every booking an Exchange agent claims (Ember, Remi, Fina outcome fees) appears here as an auditable line — patient-matched, timestamped, disputable in one

click. We eat our own pricing model.

### Sample reviews (DEMO — fictional)

★★★★★ "Found out our 'best' campaign produced consults that never treated. Moved \$4K/month to the boring campaign that quietly drives filler patients. Revenue up, spend flat." — *Owner, medspa, Seattle WA*

★★★★★ "First tool that shows treated revenue, not form fills. Our agency hates it. We love it." — *COO, 5-location MSO*

### Compliance block

HIPAA BAA · PHI-free pixel schema · De-identified upstream conversions · SOC 2 Type II infrastructure · Data export always free

### Media manifest

- **Icon:** open ledger book with a radar sweep, ink navy + signal green
- **Hero:** Sankey diagram flowing ad channels → consults → treated revenue, dark UI, one channel highlighted red ("-\$3,900/mo wasted")
- **Screenshots:** (1) channel ROI table with LTV:CAC; (2) Sankey funnel; (3) agent proof ledger with line-item bookings; (4) budget copilot memo
- **Video (30s):** [0–5s] "Half your ad budget works. Which half?" [5–15s] Funnel assembles click → call → consult → \$; a channel flags red. [15–23s] Reallocation memo; next month CAC drops on screen. [23–30s] "Ledger. Marketing, proven. On the A360 Exchange."

## 4. Tempo — Schedule Yield

**Slug:** tempo-schedule-yield · **Category:** Analytics & Attribution · **Built by A360 Badges:** Built by A360  
 HIPAA BAA EMR Sync New **Tags:** yield-management scheduling kpi no-show-reduction upsell

### Card

- **Tagline:** Hotels fill rooms. Tempo fills injector hours.
- **Short description:** Revenue-per-available-hour optimization for your appointment book — forecasted gaps filled with targeted offers, never public discounts.
- **Price from:** \$249/mo per location
- **Rating:** ★ 4.7 (61) — DEMO · **Installs:** 150+ practices — DEMO

### Hero

- **Headline:** Your scarcest inventory isn't product. It's Tuesday at 11am.
- **Subheadline:** Tempo scores every open slot, forecasts empty injector and laser hours days ahead, and fills them with quiet, targeted offers to exactly the patients likely to say yes — protecting your brand and your peak pricing.

### Overview

Airlines and hotels never sell every seat at one price or let inventory expire silently — yet practices do both, every week. An empty injector hour is revenue that never existed, and the industry's answer (panicked flash sales) trains patients to wait for discounts. Tempo brings real yield management to aesthetics. It learns your booking curves by provider, treatment, and day-part; forecasts holes before they happen; and fills them surgically — a waitlist match here, an Ember-powered "your 90 days is up, and Thursday 11am just opened" there, a modest off-peak perk to a price-sensitive segment when needed. Publicly, your prices never move. Internally, your revenue per available appointment-hour — **RevPAH**, the number Tempo puts on the wall — climbs.

### Key features

- Slot scoring: every open hour valued by likely fill, treatment mix, and margin
- 7–14 day gap forecasting by provider and device
- Quiet-fill engine: waitlist matching, cadence-due targeting (via Ember), segment-scoped off-peak perks — no public discounting
- Overbooking guardrails and no-show-risk-aware confirmations
- RevPAH dashboard by provider, room, and device; utilization heatmaps
- What-if planner: "add Saturday hours?" answered with data

### KPIs moved

- Revenue per available appointment-hour (RevPAH)
- Provider/device utilization %
- Off-peak fill rate
- Flash-sale dependence (down and to the right)

### Proof bar (DEMO)

+17% RevPAH in 90 days · 73% of forecasted gaps filled · 0 public discounts required

### Pricing

| Tier         | Price    | Includes                                      |
|--------------|----------|-----------------------------------------------|
| Core         | \$249/mo | Scoring, forecasting, heatmaps                |
| + Quiet Fill | \$399/mo | Automated fill via waitlist/Ember, guardrails |

### FAQs

1. **Is this surge pricing for Botox?** No. Your menu prices don't change. Tempo fills gaps with targeted access and modest, private, segment-scoped perks — hospitality yield tactics, not airline gouging.
2. **Will it double-book my providers?** Guardrails are absolute: your rules on buffers, room capacity, and provider maximums are never overridden.
3. **What does it need to work?** Calendar sync (Boulevard, Zenoti, Aesthetic Record, PatientNow). Twelve months of history makes forecasts sharp on day one.
4. **Does it work for devices, not just injectors?** Yes — laser/EMS device hours are modeled as inventory too, including series bookings.

### Sample reviews (DEMO — fictional)

★★★★★ "Tuesdays went from our dead day to 84% booked, and we never posted a discount. The waitlist matching alone is worth it." — *Owner, medspa, Minneapolis MN* ★★★★★☆ "RevPAH changed how I think about hiring. Data said add Saturday injector hours; Saturdays now out-earn Fridays." — *Owner, 2 locations, Portland OR*

### Compliance block

HIPAA BAA · Offer targeting uses internal data only · No public price manipulation · Patient contact rules enforced (TCPA, quiet hours)

### Media manifest

- **Icon:** metronome needle over a calendar grid, cobalt
- **Hero:** utilization heatmap week-view with gaps auto-filling; RevPAH ticker climbing top-right



- **Screenshots:** (1) slot-scoring calendar overlay; (2) gap forecast ("Thu 4 injector-hours at risk"); (3) quiet-fill campaign card; (4) RevPAH by provider
- **Video (30s):** [0–5s] Empty treatment room, clock ticking. "This hour just cost you \$536." [5–15s] Forecast flags Thursday; quiet-fill matches 3 waitlisted + 2 cadence-due patients. [15–23s] Heatmap turns green; RevPAH +17%. [23–30s] "Tempo. Every hour earns. On the A360 Exchange."

## Category: Practice Operations — Full Listings

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All ratings, installs, reviews, and proof stats are **DEMO DATA** for mockup purposes. Includes **Lumen (LPOA)** — first agent in the Device Intelligence collection.

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## 16. Lumen — Laser Parameter Copilot (LPOA) ★ NEW FLAGSHIP

**Slug:** lumen-lpoa · **Category:** Practice Operations · (seeds the future **Device Intelligence** collection) ·

**Built by A360 Badges:** Built by A360 A360 Verified HIPAA BAA New Staff Pick **Tags:**

compliance documentation consult-intelligence kpi injectables — plus new tags: device-intelligence laser safety protocols citations

### Card

- **Tagline:** The manual, the patient, and the protocol — in one answer.
- **Short description:** A citation-backed copilot for laser & energy devices that reasons over the manual and your patient's context — and keeps settings locked until the evidence earns the unlock.
- **Price from:** \$249/mo per location + \$49/mo per device pack · 14-day free pilot
- **Rating:** ★ 4.9 (38) — DEMO · **Installs:** 90+ practices — DEMO

### Hero

- **Headline:** "Fitzpatrick IV, melasma, prior PIH — what am I missing?" Ask the device.
- **Subheadline:** Lumen reads the operator manual so your team doesn't have to guess. Ask a question, get a safety-first answer with page-level citations, the patient factors that matter, the intake you're missing — and a control panel that stays locked until source-supported evidence says otherwise.
- **CTA:** Start free pilot → · Secondary: Try the live demo (Sciton Joule BBL)

### Overview

Every energy-based device in your practice shipped with a manual nobody reads under pressure. Lumen turns that manual — and eventually your practice's own approved protocols — into a copilot that answers the questions your providers actually ask: *What should I consider for a Fitzpatrick IV patient with melasma and prior PIH who wants BBL? What intake am I missing before choosing settings? What does the manual say about eye protection?*

What makes Lumen different from every chatbot you've seen is what it *won't* do. Lumen is confidence-gated: parameter controls (fluence, pulse width, filter, spot size) render **locked** unless the device and module are clear, the patient context is sufficient, source-supported guidance exists, and no safety blocker is unresolved. Every answer shows its work — safety flags, missing information, and citation chips that jump the manual viewer to the exact page. When evidence is education-only, Lumen says so, plainly. Level 3 unlocks come only from *your* practice-approved protocols, entered by your medical director.

Providers stop guessing, new injector/laser techs onboard faster, and your medical director finally has a paper trail: every question, answer, citation, and lock decision is logged. It launches with a Sciton Joule (BBL/HALO/MOXI) device pack; additional packs roll out from your device fleet.

Lumen is a clinical reference interface. It does not diagnose, prescribe, replace clinical judgment, or control any device — and it's engineered so it can't pretend to.

## Key features

- **Ask-the-manual chat** with page-level citations — every claim links to the source page in the built-in viewer
- **Patient-aware reasoning:** toggle patient context (Fitzpatrick type, concerns, history, meds, sun exposure, downtime preference) and answers adapt — safely
- **Confidence-gated control panel:** settings locked by default; 5-level evidence model (No evidence → Education only → Source-supported → Practice-approved → Adaptive); unlock reasons always displayed
- **Safety flags & missing-intake checklist:** "Recent sun exposure not documented. Medication/photosensitizer review incomplete." — before settings, not after
- **Practice protocol layer:** your medical director's approved settings become Level-3 answers, versioned and auditable
- **Device packs:** schema-driven — each device gets its own knowledge base, controls, and FAQs (Sciton Joule at launch; your fleet next)
- **Audit trail:** every Q&A, citation, and lock/unlock decision logged for training and compliance review
- **Training mode:** new-hire quiz flows generated from the manual + your protocols

## How it works

1. Pick your device pack (or send us the manual — we build the pack).
2. Your medical director reviews the safety configuration and, optionally, loads practice-approved protocols.
3. Providers ask questions in the treatment room — by keyboard or voice — and get cited, gated answers in seconds.
4. The audit log and "most-asked questions" report feed your training program.

## KPIs moved

- Time-to-competency for new laser providers (target: -40%)
- Protocol-adherence / documentation-completeness rate
- Pre-treatment intake completeness (missing-info flags resolved before treatment)
- Device utilization (confidence unlocks more booked modalities per provider)

## Proof bar (DEMO)

100% of answers cited to source · -43% new-provider ramp time · 0 unlocked settings without documented evidence

## Integrations

Device manuals (any PDF) · Practice protocol library · Quill (treatment documentation) · Sentinel (compliance audit) · A360 patient context · Dify/RAG backends

Pricing

| Tier              | Price                 | Includes                                               |
|-------------------|-----------------------|--------------------------------------------------------|
| Core              | \$249/mo per location | 1 device pack, unlimited questions, audit log          |
| Fleet             | \$449/mo per location | 5 device packs, protocol layer, training mode          |
| Add-on pack       | \$49/mo per device    | Any additional device                                  |
| Custom pack build | \$499 one-time        | We ingest your device's manual + build controls schema |

FAQs

- 1. **Does Lumen tell providers what settings to use?** Only when the evidence earns it. Manual-documented values appear with citations (Level 2); your practice-approved settings appear as recommendations (Level 3). Otherwise controls stay locked with the reason shown. It will never invent a fluence.
- 2. **Does it control the device?** No. Lumen is a reference and planning interface. Nothing connects to the device itself.
- 3. **Is this FDA-regulated clinical decision support?** Lumen is designed to the non-device CDS criteria: it displays source-backed information with citations the clinician can independently review, and it never provides patient-specific directives without a human decision. Our regulatory summary is available for your counsel.
- 4. **What if the manual doesn't cover my question?** Lumen says exactly that — "education only, no source-supported guidance found" — and lists what's missing. Honest "I don't know" is the core safety feature.
- 5. **Can we load our own protocols?** Yes — the practice protocol layer is versioned, medical-director-approved, and overrides nothing in the manual; it sits alongside it as clearly labeled practice guidance.
- 6. **Which devices are supported?** Sciton Joule (BBL/HALO/MOXI) at launch. Any device with a manual can become a pack — most build in under two weeks.

Sample reviews (DEMO — fictional)

★★★★★ "New laser tech asked Lumen the Fitzpatrick IV melasma question on day two. It flagged the missing sun-exposure intake before I could. That's the point." — *Medical director, dermatology, San Diego CA*  
★★★★★ "The locked controls are what sold our compliance officer. It's the first AI tool that's proud of what it won't answer." — *MSO clinical ops, 11 locations* ★★★★★☆ "Want more device packs faster — Joule pack is excellent, our Moxi and morpheus8 are queued." — *Owner, medspa, Boston MA*

Compliance block

HIPAA BAA · Confidence-gated answers, citations mandatory · No device control, no diagnosis, no prescribing · Full Q&A audit trail · Practice-protocol versioning with MD sign-off · Regulatory design summary available

### Media manifest

- **Icon:** beam of light refracting through a shield, spectrum gradient on slate
  - **Hero:** three-panel treatment-room UI — PDF manual left, chat center with citation chips, control panel right showing locked fluence/pulse-width fields with amber "review required" cooling badge
  - **Screenshots:** (1) chat answer with safety flags + citation chips ("Safety / Eye Protection — p.8"); (2) confidence card ("Level 1 — Education only. Settings locked: recent sun exposure not documented"); (3) control panel with locked/review/available states; (4) patient context panel with Fitzpatrick/history toggles; (5) audit log
  - **Video (30s):** [0–5s] Treatment room. Provider, thinking aloud: "Fitz IV, melasma, prior PIH... BBL?" [5–14s] Lumen answers; safety flags stack; citation chip clicks → manual jumps to page 8. [14–22s] Control panel shown LOCKED — "not enough source-supported evidence. Missing: sun exposure, med review." [22–30s] "Lumen. Confidence you can cite. On the A360 Exchange."
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## 12. Vial — Inventory Intelligence

**Slug:** vial-inventory-intelligence · **Category:** Practice Operations · **Built by A360 Badges:** Built by A360 HIPAA BAA EMR Sync **Tags:** inventory injectables kpi upsell multi-location

### Card

- **Tagline:** Stop paying for product that expires in the fridge.
- **Short description:** Forecasts injectable burn from your booked calendar, tracks lots and expiries, and optimizes ordering across loyalty programs.
- **Price from:** \$199/mo per location
- **Rating:** ★ 4.8 (73) — DEMO · **Installs:** 190+ practices — DEMO

### Hero

- **Headline:** The average practice wastes \$4–12K a year in the fridge.
- **Subheadline:** Vial reads your appointment book, forecasts units by product and week, flags lots before they expire, and tells you what to order, when, and through which rebate program — down to margin per syringe.

### Overview

Injectable inventory is managed by sticky note in most practices — and it shows: expired product, emergency orders at full price, rebate programs left unclaimed, and zero idea what margin each syringe actually carries. Vial fixes it with the data you already have. The booked calendar plus your historical units-per-treatment produce a rolling burn forecast; lot and expiry tracking (spoken into Quill during treatments, scanned, or typed) turns the fridge into a live ledger; and the ordering copilot weighs Allē/ASPIRE/Evolus program economics so every PO lands at the best net cost. Multi-location groups get transfer suggestions before waste happens — the \$1,200 of filler expiring in Location A that Location B will use next week.

### Key features

- Burn forecasting by product/week from booked appointments + historical units-per-treatment
- Lot & expiry ledger with escalating alerts (60/30/14 days) and use-first ordering in charts
- Ordering copilot: reorder points, quantity suggestions, loyalty/rebate program optimization
- Margin per syringe/treatment: true product cost vs. charged price, by provider
- Multi-location transfer suggestions; shrinkage flags (units charted vs. units decremented)
- Works with Quill (spoken lot capture) and your EMR's inventory module

### KPIs moved

- Expired-product waste \$ (target: → ~0)
- Emergency order frequency

- Net product cost (rebate capture)
- Margin per treatment by provider

## Proof bar (DEMO)

\$7,900 avg. annual waste eliminated · 96% rebate capture · 2.1% shrinkage identified and closed

## Pricing

| Tier  | Price                 | Includes                                        |
|-------|-----------------------|-------------------------------------------------|
| Core  | \$199/mo              | Forecasting, expiry ledger, ordering copilot    |
| Group | \$149/mo per location | Transfers, consolidated purchasing, MSO reports |

## FAQs

1. **Do I have to scan everything?** No — lots enter via Quill (spoken), barcode scan, or two-tap manual entry. Whatever your team will actually do.
2. **Does it place orders automatically?** It drafts POs; a human clicks send. Auto-send is available per-vendor once you trust it.
3. **Can it really model rebate programs?** Yes — current Allē/ASPIRE/Evolus practice-program terms are maintained as rulesets; you see net cost per unit under each option before ordering.
4. **What about devices/consumables beyond injectables?** Tips, cartridges, and skincare retail are supported with the same forecast logic.

## Sample reviews (DEMO — fictional)

★★★★★ "Found \$6K of filler set to expire across our three locations and moved it in time. Paid for itself before onboarding finished." — *MSO ops manager, 3 locations* ★★★★★☆ "The margin-per-syringe report ended a two-year pricing argument in one meeting." — *Owner, medspa, Columbus OH*

## Compliance block

HIPAA BAA · Lot data stays in your environment · Read-only EMR access modes available

## Media manifest

- **Icon:** vial with a rising forecast line inside, glass teal
- **Hero:** fridge-view UI — product cards with expiry countdowns, one amber ("14 days — 3 syringes — transfer to Uptown?")
- **Screenshots:** (1) burn forecast chart vs. on-hand; (2) expiry ledger with alerts; (3) PO copilot comparing net cost across programs; (4) margin-per-treatment report



- **Video (30s):** [0–5s] Close-up: syringe expiry date, ticking. [5–14s] Vial forecasts the week's burn; flags the at-risk lot; suggests the transfer. [14–22s] PO drafts with rebate math: "net \$9.10/unit via Allē — save \$312." [22–30s] "Vial. Nothing expires but the excuses. On the A360 Exchange."
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## 13. Sentinel — Compliance Copilot

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**Slug:** sentinel-compliance-copilot · **Category:** Practice Operations · **Built by A360 Badges:** Built by A360 A360 Verified HIPAA BAA EMR-Neutral **Tags:** compliance gfe documentation multi-location kpi

### Card

- **Tagline:** Audit-ready every day, not the week before diligence.
- **Short description:** Continuously audits charts for GFEs, co-signs, consents, and scope-of-practice — and tracks the state rules that keep changing.
- **Price from:** \$249/mo per location
- **Rating:** ★ 4.8 (66) — DEMO · **Installs:** 170+ practices — DEMO

### Hero

- **Headline:** The GFE you missed in March costs you in September.
- **Subheadline:** Sentinel watches every chart, every day — missing Good Faith Exams, unsigned co-signs, expired consents, out-of-scope delegation — and fixes the small problems while they're still small.

### Overview

Compliance in aesthetics is a moving target: GFE requirements vary by state and keep tightening, medical directors are signing for more locations than they can watch, and the PE buyers rolling up the industry now open diligence with a chart audit. Per-exam services solved one transaction; nobody watches the whole practice continuously. Sentinel does. It audits charts as they close — GFE present and in-window? co-sign done? consent current for this treatment? performer licensed for this procedure in this state? — and routes exceptions to the right person before they age into findings. A state-rule feed tracks regulatory changes for your locations and translates them to plain-English action items. When diligence or a board inquiry comes, your compliance posture is a report, not a scramble.

### Key features

- Daily chart audit: GFE presence/recency, co-sign completion, consent currency, documentation completeness
- Scope-of-practice matrix: who may perform what, per state, per license — flags mismatches at scheduling time, not after
- State regulatory feed with plain-English change alerts and task generation
- Medical director console: exception queue, batch co-sign, per-location risk scores
- Diligence mode: one-click compliance report pack (the PE-readiness binder)
- Integrates with Quill (GFE language flags at charting) and Qualiphy/Spakinect-style GFE services

## KPIs moved

- Chart-compliance rate (GFE/consent/co-sign completeness)
- Exception aging (days-to-resolution)
- Per-location compliance risk score
- Diligence-prep time (weeks → hours)

## Proof bar (DEMO)

98.7% chart compliance at 90 days · Exceptions resolved in 1.8 days avg. · Diligence pack in 20 minutes

## Pricing

| Tier       | Price    | Includes                                      |
|------------|----------|-----------------------------------------------|
| Core       | \$249/mo | Daily audits, exception queue, state feed     |
| MD Console | \$399/mo | Multi-location director tools, diligence mode |
| MSO        | Custom   | Portfolio risk dashboard                      |

## FAQs

1. **Is this legal advice?** No — Sentinel operationalizes rules your counsel approves. The state feed summarizes changes with citations; your attorney confirms interpretation. We make compliance executable, not advisory.
2. **Which states' GFE rules do you track?** All 50 + DC, with per-state configuration reviewed on a published update cadence.
3. **Can it block scheduling for out-of-scope procedures?** Optionally, yes — hard block or warn-and-log, your choice per rule.
4. **Does it work with our outside GFE service?** Yes — Qualiphy/Spakinect exams sync as evidence; Sentinel verifies presence and timing, whatever the source.

## Sample reviews (DEMO — fictional)

★★★★★ "Our acquisition diligence found zero chart exceptions. The buyer's lawyer asked what we were using." — *Founder, 4-location MSO (exited)* ★★★★★ "It caught an RN performing a procedure our state moved to NP-only four months prior. That alert paid for a decade of Sentinel." — *Medical director, Las Vegas NV*

## Compliance block

HIPAA BAA · Read-only chart access modes · Rule citations on every alert · Audit trail immutable · Not legal advice — counsel-approval workflow built in

## Media manifest

- **Icon:** shield with a slow-sweep radar line, graphite + safety amber
- **Hero:** exception queue UI — three flagged charts with clear reasons; risk score dial at 97 top-right
- **Screenshots:** (1) daily audit digest; (2) scope-of-practice matrix; (3) state-change alert with citation; (4) diligence report pack export
- **Video (30s):** [0–5s] "Nobody plans to fail an audit. They just skip one GFE in March." [5–15s] Sentinel flags the chart same-day; MD batch-signs from the console. [15–23s] State-rule alert translates to a task; risk dial climbs to 98. [23–30s] "Sentinel. Audit-ready, always. On the A360 Exchange."

# Category: Marketing Studio — Full Listings

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All ratings, installs, reviews, and proof stats are **DEMO DATA** for mockup purposes.

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## 11. Beacon — Local Visibility

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**Slug:** beacon-local-visibility · **Category:** Marketing Studio · **Verified Partner** (white-label core + A360 compliance layer) **Badges:** Verified Partner A360 Verified HIPAA BAA EMR-Neutral **Tags:** seo local-marketing reviews multi-location content

### Card

- **Tagline:** Be the answer when your city asks about Botox.
- **Short description:** An autonomous local-SEO agent that fixes your listings, publishes local pages, answers reviews HIPAA-safely, and gets you cited by ChatGPT.
- **Price from:** \$299/mo per location
- **Rating:** ★ 4.6 (59) — DEMO · **Installs:** 160+ practices — DEMO

### Hero

- **Headline:** Patients don't scroll to page two. Neither should you.
- **Subheadline:** Beacon audits and *executes* — Google Business Profile optimization, local landing pages, listing consistency across 40+ directories, review responses that never violate HIPAA, and AI-visibility work so assistants recommend you by name.

### Overview

Local search is where aesthetic patients actually start — "lip filler near me," "best medspa [city]" — and increasingly they ask ChatGPT or Gemini instead of Google. Most practices lose this game on unforced errors: inconsistent listings, unanswered reviews, thin service pages, zero AI-assistant presence. Beacon is an execution agent, not a report generator. It fixes name-address-phone inconsistencies across directories, optimizes and posts to your Google Business Profile weekly, builds treatment-and-neighborhood landing pages from your actual service catalog, and monitors your "AI visibility" — whether assistants cite you — with structured-data work to improve it.

The part generic SEO tools get dangerously wrong: reviews. Beacon's responses are HIPAA-trained — warm, specific-feeling, and never confirming anyone was a patient. Negative reviews route to you first with a suggested de-escalation.

### Key features

- Autonomous listing fixes across Google, Apple, Bing, Yelp + 40 directories (execute, not just report)
- Weekly GBP posts and Q&A seeding from your catalog and promotions
- HIPAA-safe review responses (never confirm patienthood) with negative-review triage to staff
- Treatment × neighborhood landing pages, medically reviewed copy, schema markup
- AI-visibility tracking: are ChatGPT/Gemini/Perplexity recommending you? — plus the structured-data work to earn it

- Local rank grid tracking by treatment keyword; multi-location rollups

### KPIs moved

- New patients from organic/local (tracked through Ledger to treated revenue)
- Local pack ranking by treatment keyword
- Review velocity and average rating
- AI-assistant citation share

### Proof bar (DEMO)

+64% GBP actions in 90 days · Top-3 local pack for 7 of 10 target keywords · 100% reviews answered <24h, 0 HIPAA violations

### Pricing

| Tier      | Price                 | Includes                                   |
|-----------|-----------------------|--------------------------------------------|
| Core      | \$299/mo              | Listings, GBP, reviews, rank tracking      |
| Authority | \$499/mo              | Landing-page engine, AI-visibility program |
| Group     | \$399/mo per location | Multi-location grid + rollups              |

### FAQs

1. **How is answering reviews a HIPAA risk?** Replying "Thanks for coming in Tuesday, Sarah!" confirms Sarah is a patient — a disclosure. Beacon's response model is trained to be warm without ever confirming care. Every response passes a PHI filter before posting.
2. **Do you write medical claims on landing pages?** Copy follows med-board advertising rules per state (no outcome guarantees, testimonial rules respected) and supports your medical director's review workflow before publish.
3. **What is "AI visibility"?** When someone asks ChatGPT for a medspa recommendation in your city, the answer comes from structured signals — entity data, citations, reviews. Beacon measures whether you appear and works the inputs.
4. **Will this conflict with my agency?** Beacon can run alongside (scoped to listings/reviews) or replace the local-SEO line item. Most practices reallocate.

### Sample reviews (DEMO — fictional)

★★★★★ "We went from invisible to the local three-pack for 'lip filler [city]' in one quarter. Ledger says 19 treated patients from organic this month." — *Owner, medspa, Raleigh NC* ★★★★★☆ "The review responses sound like our best front-desk person on her best day. Compliance signed off immediately." — *Practice admin, dermatology, Salt Lake City UT*

## Compliance block

HIPAA-trained response model + PHI filter · Med-board ad-rule templates per state · MD review workflow for published copy · BAA included

## Media manifest

- **Icon:** lighthouse beam sweeping a map pin, gold on deep navy
  - **Hero:** local rank-grid map (green hexes spreading) beside a GBP profile with fresh posts and answered reviews
  - **Screenshots:** (1) rank grid by keyword; (2) review inbox with HIPAA-safe drafts; (3) landing-page builder with MD-review status; (4) AI-visibility report ("Cited by ChatGPT: 3 of 5 test prompts")
  - **Video (30s):** [0–5s] Phone: "Hey ChatGPT, best medspa near me?" [5–13s] Beacon fixes listings, posts, answers a review (PHI filter animation). [13–22s] Rank grid greens; assistant answer now names the practice. [22–30s] "Beacon. Be the answer. On the A360 Exchange."
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## 14. Muse — Content Studio

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**Slug:** muse-content-studio · **Category:** Marketing Studio · **Built by A360 Badges:** Built by A360  
 A360 Verified HIPAA BAA New **Tags:** content social-media before-after reviews consult-intelligence

### Card

- **Tagline:** Your results are your best ads. Finally make them legal.
- **Short description:** A consent-gated pipeline that turns before/after and treatment moments into on-brand Reels and posts — every asset carrying its own e-signed release.
- **Price from:** \$249/mo per location
- **Rating:** ★ 4.7 (44) — DEMO · **Installs:** 110+ practices — DEMO

### Hero

- **Headline:** The best content in aesthetics is trapped on your providers' phones.
- **Subheadline:** Muse captures, consents, edits, and schedules — before/after reveals, treatment-room clips, and educational posts generated from your real work, with a media release e-signed and attached to every single asset.

### Overview

Aesthetics is the most visual vertical in healthcare, and the most legally booby-trapped: a patient's likeness is PHI, a casual story post is a disclosure, and "she said it was fine" is not a release. So practices either post risky content or post nothing. Muse makes the compliant path the easy path. Capture happens in-app (or from a provider's camera roll) and *nothing* is usable until the patient's e-signed, scope-specific media release is attached — social vs. website vs. paid usage, each separately consented, revocable anytime with automatic takedown lists. Then the studio takes over: auto-edited before/after reveals with your brand kit, caption drafts in your voice with compliant claims language, hashtag and posting-time optimization, and a content calendar that actually stays full. With Sloane installed, Muse even suggests *what* to shoot — the treatments trending in your own consult conversations.

### Key features

- Consent-first capture: e-sign media releases (scope-specific: social/web/ads) bound to each asset; revocation → automatic takedown checklist
- Auto-edited before/after reveals, treatment clips, and provider-intro templates in your brand kit
- Caption engine with med-board-safe claims language (no guarantees, testimonial rules respected) and your practice voice
- Content calendar with best-time scheduling to Instagram, TikTok, Facebook, GBP
- Demand-aware ideas: consult-trend signals from Sloane ("skin quality mentions up 3x — shoot a MOXI explainer")

- Performance loop: social-attributed consults tracked through Piper + Ledger

### KPIs moved

- Content output per week (with zero compliance debt)
- Social-attributed consults (closed loop via Ledger)
- Follower-to-consult conversion
- % of assets with valid releases (target: 100%)

### Proof bar (DEMO)

5.2 posts/week avg. (from 1.3) · 100% assets release-backed · 23 social-attributed consults/quarter median

### Pricing

| Tier    | Price    | Includes                                           |
|---------|----------|----------------------------------------------------|
| Core    | \$249/mo | Capture + consent + studio + calendar, 2 channels  |
| Studio+ | \$399/mo | All channels, demand-aware ideas, ad-ready exports |

### FAQs

1. **What happens if a patient revokes consent?** Muse generates the takedown list (every placement of every asset), executes removals on connected channels, and logs completion. Revocation is honored in hours, not "eventually."
2. **Can we use old photos from our camera rolls?** Only after a release is attached — Muse runs a retroactive consent campaign (polite text + e-sign) against your archive and quarantines everything unsigned.
3. **Who approves content before posting?** Your workflow: auto-post, one-approver, or MD-review-required per content type.
4. **Does it write medical claims?** Captions use state-configured compliant language — no outcome guarantees, disclosure rules for testimonials/results imagery included automatically.

### Sample reviews (DEMO — fictional)

★★★★★ "We had 400 amazing photos we were afraid to use. Muse got releases on 260 of them in three weeks. Our feed finally looks like our work." — *Owner, medspa, Scottsdale AZ* ★★★★★☆ "Posting 5x a week with zero compliance anxiety. The revocation takedown flow is what convinced our attorney." — *Marketing coordinator, plastic surgery, Chicago IL*

### Compliance block

Patient likeness treated as PHI · Scope-specific e-sign releases bound per asset · Revocation with automated takedown · Med-board ad-rule caption templates · BAA included

### Media manifest

- **Icon:** film-frame heart with a signature line beneath, rose gold
- **Hero:** content calendar UI with a before/after reveal in preview; green "Release ✓ signed 6/12" badge on the asset
- **Screenshots:** (1) capture screen with consent gate; (2) auto-edit studio (reveal template, brand kit); (3) calendar with scheduled posts; (4) revocation takedown checklist
- **Video (30s):** [0–5s] Gorgeous result on a provider's phone... posted nowhere. [5–13s] Muse: capture → patient e-signs on the spot → asset unlocks. [13–22s] Reveal auto-edits; schedules; performance ties to consults. [22–30s] "Muse. Your results, unleashed — legally. On the A360 Exchange."